NCE EXAM PREP

Practice Exam 28

Domain 4

Provision of Counseling Interventions

34 Practice Questions | 45 minutes

A, B, C answer choices

Answer Key and Explanations included

> **Instructions.** Select the single best answer for each question. Record your answers separately. This domain practice set is drawn from Full-Length Practice Exam 05. Use the suggested 45-minute practice timer. Do not consult Part B until you complete Part A. These domain timings are editorial practice allocations, not official NBCC section limits.

Independent educational practice material. Not affiliated with, sponsored, endorsed, or approved by NBCC. These are original practice questions, not actual NCE items.

PART A — QUESTIONS

Choose one answer (A, B, or C) per question.

Question 1

> After surviving a highly publicized assault, a client receives repeated calls from reporters and distant acquaintances. The client is safe but feels unable to decide whom to answer and says, “Everyone wants part of the story.”

Which intervention best supports the client?

A) Encourage the client to tell the full story once publicly so requests will stop.

B) Advise ignoring every contact because any response will worsen distress.

C) Help the client define privacy boundaries, choose whether to designate one trusted contact, and rehearse a brief response to unwanted requests.

Question 2

> A college student has been outed as gay by a peer without consent. The student is safe but fears that family members will learn through social media and says, “I need to decide who hears this from me.”

What should the counselor do?

A) Support the student in identifying safe contacts, reviewing privacy options, and deciding whether, when, and how to communicate with others.

B) Encourage immediate disclosure to all family members so the student controls the story.

C) Contact the student’s family to request that they respond supportively.

Question 3

> A client from a rural community says the phrase “mental health treatment” carries strong stigma locally and prefers to describe counseling goals as managing stress and improving relationships. The requested goals remain within counseling scope.

How should the counselor respond?

A) Use language that fits the client’s understanding while remaining transparent about the counseling service and its purpose.

B) Insist on clinical terminology so the client learns to accept a mental health identity.

C) Describe counseling as an unrelated wellness service so the client can avoid informed-consent concerns.

Question 4

> A client who stopped smoking three months ago will attend a reunion where several close friends smoke. The client wants to protect progress and is willing to plan for a possible lapse without treating it as total failure.

Which intervention is most appropriate?

A) Identify likely cues, rehearse refusal and coping responses, and create a plan for resuming the goal promptly after any lapse.

B) Advise skipping every future event where smoking might occur.

C) Emphasize that one cigarette would erase the progress already made.

Question 5

> A client says, “You probably think staying in this relationship means I’m weak.” The counselor does not hold that view and is uncertain whether the statement reflects a prior experience, a current cue, or both.

Which response is most therapeutic?

A) “You’re concerned I see you as weak. What have you noticed here, and where else have you encountered that judgment?”

B) “I do not think you are weak, so you do not need to worry about being judged here.”

C) “That sounds like transference from an earlier relationship rather than something occurring between us.”

Question 6

> In an integrated-care clinic, a client has just received a serious medical diagnosis. The client is safe but says, “I heard the doctor talking, but none of it stayed in my head.” The client wants help preparing for the next medical conversation.

What is the counselor’s best intervention?

A) Interpret the difficulty remembering as denial of the diagnosis.

B) Help the client identify the most urgent questions, write them in the client’s words, and choose a support person for the next conversation.

C) Explain the medical condition so the client does not need to ask the physician again.

Question 7

Which statement best describes counselor empathy?

A) Feeling the same emotion as the client with equal intensity.

B) Agreeing that the client’s interpretation is objectively correct.

C) Seeking to understand the client’s experience from the client’s perspective and communicating that understanding.

Question 8

> A client values collective family decision making and asks to bring a trusted aunt to one session to help practice a new stress-management routine at home. The client wants to decide what the aunt hears and does.

What should the counselor do?

A) Clarify the client's goals and consent, then include the aunt in the agreed skill-practice role.

B) Decline because involving family would undermine the client's individual autonomy.

C) Give the aunt full information so the aunt can monitor the client's compliance.

Question 9

> A parent and 13-year-old want to reduce late-night gaming on school nights. They agree that homework completion and a consistent shutdown time are reasonable targets, and the adolescent identifies weekend gaming time as a meaningful privilege.

Which intervention is most appropriate?

A) Remove all gaming indefinitely so the consequence is strong enough to change behavior.

B) Let the parent decide whether the adolescent was cooperative enough each day.

C) Create a specific, observable agreement linking the defined targets to the agreed privilege and review the data together.

Question 10

Which description best reflects healthy group cohesion?

A) Members avoid disagreement to preserve a consistently comfortable atmosphere.

B) Members depend on the leader to resolve every interpersonal tension.

C) Members experience belonging and enough trust to engage honestly, including working through disagreement.

Question 11

> Family members disagree about how to mark the anniversary of a grandparent's death. Some want a large gathering, while another wants a private day of reflection. Each describes the other approach as disrespectful.

What should the family counselor do?

A) Recommend the larger ritual because shared remembrance promotes family cohesion.

B) Invite each person to explain the meaning of the preferred ritual and help the family consider ways to honor more than one form of grief.

C) Ask the most recently bereaved member to decide for everyone.

Question 12

> A graduate student understands course material but freezes during timed examinations. The student reports rushing, losing track of instructions, and abandoning coping skills once the timer starts.

Which intervention best matches the problem?

A) Rehearse coping skills during gradually timed practice tests and review which responses improve performance.

B) Focus only on studying more content so confidence increases before the next examination.

C) Advise avoiding all timed practice because it may reinforce test anxiety.

Question 13

> A client trying to stop vaping notices cravings rise and fall during work breaks. The client assumes every craving will keep increasing until vaping occurs and is willing to practice observing one in session.

Which intervention best fits?

A) Distract the client immediately so no awareness of craving occurs.

B) Guide the client to observe the craving's sensations and changing intensity without acting on it, like riding a wave.

C) Reassure the client that cravings are harmless and will disappear permanently after one practice.

Question 14

> A client reports current suicidal intent and access to a firearm at home. The agency's emergency protocol requires continuous supervision and immediate emergency evaluation when intent and lethal-means access are present.

What should the counselor do?

A) Schedule a next-day appointment after obtaining a verbal promise that the firearm will not be used.

B) Ask the client to drive home, secure the firearm, and then go to an emergency department.

C) Remain with the client, activate the emergency protocol, and communicate the risk information needed for a safe evaluation.

Question 15

> A client has difficulty distinguishing numerical ratings such as 6 versus 7 on a 0-to-10 distress scale. The client can reliably describe distress as mild, moderate, strong, or overwhelming.

How should the counselor adapt monitoring?

A) Continue the numerical scale until repeated practice improves precision.

B) Use the client's descriptive anchors consistently and connect each level with observable signs and coping choices.

C) Stop monitoring distress because a standardized number is required for meaningful comparison.

Question 16

> A client tells the counselor, “I wish you were my parent. You actually listen.” The counselor recognizes the statement as meaningful but cannot take on a parental role.

What is the counselor’s best response?

A) Reassure the client that the counselor cares in the same way a parent would.

B) Redirect to the client’s family history without discussing the statement about the counselor.

C) Explore what the wished-for relationship represents while maintaining the counselor’s therapeutic role.

Question 17

> In couple counseling, one partner reveals undisclosed credit-card debt. The other demands that the counselor identify who is “the irresponsible one,” while the disclosing partner becomes defensive.

What is the counselor’s best intervention?

A) Slow the exchange, establish a structure for factual disclosure, and help the partners examine the secrecy-criticism cycle without assigning a global label.

B) Agree that hiding debt is irresponsible so the couple can begin from a shared reality.

C) Postpone all discussion until the counselor has independently reviewed the couple’s financial records.

Question 18

What is shaping in a behavioral intervention?

A) Removing all reinforcement until the complete target behavior occurs.

B) Reinforcing successive approximations that gradually move toward the target behavior.

C) Explaining the target behavior repeatedly until the client performs it correctly.

Question 19

> An outpatient client reports almost no sleep for three nights, speaks rapidly, shifts topics, and describes plans to spend all savings on a new business that must launch today. Agency policy calls for urgent same-day evaluation when marked activation and impaired judgment emerge.

What should the counselor do?

A) Explore the business plan in detail to determine whether it is financially realistic.

B) Schedule an extra session later in the week and assign a sleep diary.

C) Explain the concern, arrange the urgent evaluation according to policy, and remain engaged through the transfer of care.

Question 20

> In a support group, a member describes a recent psychiatric hospitalization. Another member begins giving specific advice about changing medication doses based on personal experience.

What should the group leader do?

A) Allow the advice because lived experience is the group's primary therapeutic resource.

B) Interrupt the medication advice, restate the group's scope, and redirect members toward emotional support and questions for qualified prescribers.

C) End the discussion of hospitalization so medical issues do not enter the group.

Question 21

> A client says, "Whenever I talk about something painful, you turn it into a worksheet. Sometimes I need you to stay with what I'm feeling."

Which counselor response is most appropriate?

A) "Worksheets are evidence-based, so we should continue using them even when they feel uncomfortable."

B) "We can stop all structured interventions if that is what you prefer."

C) "I hear that I've moved to tasks before you felt understood. Let's slow down now, and I'd like to know what staying with you would look like."

Question 22

> A client speaks continuously for most of the session, moving through many details without pausing. With ten minutes left, the client says, "I'm not sure I'm getting anywhere."

What is the counselor's best response?

A) Ask permission to pause, summarize the central themes heard, and collaborate on where to focus the remaining time.

B) Continue listening without interruption because stopping the client would be disrespectful.

C) Select one detail and interpret its unconscious meaning before the session ends.

Question 23

> A client wants an attention-training practice but becomes restless during seated meditation and enjoys walking outdoors. The client can walk safely and is interested in a movement-based option.

How should the counselor adapt the intervention?

A) Require seated practice until restlessness stops because movement prevents mindfulness.

B) Teach mindful walking by directing attention to steps, sensations, and gently returning focus when the mind wanders.

C) Replace attention training with discussion because mindfulness is effective only when the body is still.

Question 24

> An older adult with mild cognitive impairment wandered from home and was found safely. The adult child caregiver is shaken, cannot stop checking the door, and says, “I can never sleep again.” Other family support is available tonight.

What is the counselor’s best immediate intervention with the caregiver?

A) Begin long-term work on anticipatory grief before discussing tonight’s needs.

B) Help arrange shared supervision for tonight, support brief regulation, and identify one follow-up step for a broader safety plan.

C) Reassure the caregiver that wandering probably will not happen again after such a frightening event.

Question 25

What is a warm handoff in crisis-related care?

A) An active connection to another provider or resource, ideally with the client present and next steps confirmed.

B) A written list of services given to the client to contact independently after counseling ends.

C) A referral delayed until the counselor can guarantee that the receiving service will solve the crisis.

Question 26

> Two members of a therapy group disclose that they recently began dating each other. The group agreement discourages romantic relationships among current members because of effects on safety and participation.

What is the leader’s best response?

A) Ignore the relationship unless another member formally complains.

B) Revisit the agreement, explore the relationship’s impact on the group, and address participation options without public shaming.

C) Require the couple to describe private details so the group can decide whether the relationship is harmful.

Question 27

> Adult siblings argue about care for their mother while she sits quietly in the family session. They repeatedly ask the counselor to choose a plan without asking their mother what she wants. The mother has decision-making capacity.

What should the counselor do?

A) Choose the safest plan because the siblings cannot agree.

B) Redirect the conversation to the mother’s goals and preferences, then facilitate family discussion about how to support them.

C) Ask the mother to wait outside while the siblings resolve their conflict.

Question 28

> A client begins crying and immediately says, “I’m sorry. I know you have clients with real problems.”

What is the counselor’s best response?

A) Reassure the client that other clients’ problems are not more serious.

B) Shift to a coping skill so the client does not become embarrassed by continued crying.

C) “You don’t need to apologize for your emotion here. What is happening for you as the tears come?”

Question 29

Which description best reflects therapeutic confrontation?

A) Respectfully presenting a meaningful discrepancy and inviting the client to examine it.

B) Proving that the client’s account is inconsistent so resistance decreases.

C) Warning the client about consequences in a forceful manner.

Question 30

> One partner has a chronic illness, and the other has taken on most household tasks. The caregiving partner says, “I’m exhausted,” while the ill partner hears, “You are a burden.” Both withdraw.

What is the counselor’s best intervention?

A) Ask the ill partner to complete more tasks immediately to restore equality.

B) Focus only on the caregiving partner’s burnout because it is the urgent issue.

C) Externalize the workload problem, validate both experiences, and coach a specific request-and-response conversation about support.

Question 31

> During a telehealth session, the client begins discussing a sensitive relationship issue. The counselor notices another person’s reflection moving behind the client, although the client had said the room was private at the start.

What is the counselor’s best response?

A) Continue because asking again about privacy may interrupt disclosure.

B) Pause and discreetly confirm whether the client still has the level of privacy desired before proceeding.

C) End the session immediately because privacy can no longer be guaranteed.

Question 32

> A Black woman describes being praised for “always being strong” while feeling unable to ask for help. She wonders whether counseling will also expect endurance rather than make room for vulnerability.

Which intervention is most culturally responsive?

A) Broach how racialized and gendered expectations of strength affect her, then collaborate on forms of support that fit her values.

B) Emphasize that resilience is a personal strength and encourage her to continue relying on it.

C) Avoid discussing identity because the client has not described every experience as discriminatory.

Question 33

> A client tells the counselor, “Your accent is difficult for me to understand, and I’ve been pretending because I didn’t want to offend you.”

What is the counselor’s best response?

A) Explain that the client will understand more easily after becoming familiar with the accent.

B) Suggest that the client read the counselor’s written summaries instead of discussing the issue.

C) Thank the client for saying so, invite requests for repetition, and adjust pace and wording while checking understanding.

Question 34

What does counselor congruence most directly involve?

A) Sharing every personal reaction so the client never has to guess what the counselor feels.

B) Being genuine and internally consistent while expressing only what serves the therapeutic relationship.

C) Agreeing with the client’s perspective to maintain emotional harmony.

PART B — ANSWER KEY AND EXPLANATIONS

Answers match the numbered items in Part A.

Question 1 — Correct: C

Correct Answer: Help the client define privacy boundaries, choose whether to designate one trusted contact, and rehearse a brief response to unwanted requests.

Explanation: The intervention restores control over access to the client and provides a usable boundary response without forcing public disclosure. Universal silence may be an option, but the counselor should support the client in choosing rather than prescribe it.

Question 2 — Correct: A

Correct Answer: Support the student in identifying safe contacts, reviewing privacy options, and deciding whether, when, and how to communicate with others.

Explanation: The student has experienced a loss of control over personal information, so the intervention restores choice and supports safety. Immediate broad disclosure or counselor contact with family would repeat the removal of the student's control.

Question 3 — Correct: A

Correct Answer: Use language that fits the client's understanding while remaining transparent about the counseling service and its purpose.

Explanation: Responsive language can reduce stigma and improve engagement without obscuring informed consent. Imposing an identity label is unnecessary, while disguising counseling as an unrelated service is not transparent.

Question 4 — Correct: A

Correct Answer: Identify likely cues, rehearse refusal and coping responses, and create a plan for resuming the goal promptly after any lapse.

Explanation: Relapse-prevention planning anticipates triggers, practices responses, and prevents a lapse from becoming an all-or-nothing return to smoking. Avoiding every social risk is unnecessarily restrictive, while treating one cigarette as total failure increases abstinence-violation thinking.

Question 5 — Correct: A

Correct Answer: "You're concerned I see you as weak. What have you noticed here, and where else have you encountered that judgment?"

Explanation: The counselor explores both current relational evidence and broader experience without denying the concern or imposing an interpretation. Immediate reassurance may close inquiry into a belief that is important inside and outside counseling.

Question 6 — Correct: B

Correct Answer: Help the client identify the most urgent questions, write them in the client's words, and choose a support person for the next conversation.

Explanation: Acute stress can impair retention, so organizing client-generated questions and support makes the next medical interaction more usable. Interpreting denial is premature, and the counselor should not replace qualified medical explanation.

Question 7 — Correct: C

Correct Answer: Seeking to understand the client's experience from the client's perspective and communicating that understanding.

Explanation: Empathy involves accurate perspective taking and responsive communication while preserving awareness that the experience belongs to the client. It does not require emotional equivalence or agreement with every conclusion.

Question 8 — Correct: A

Correct Answer: Clarify the client's goals and consent, then include the aunt in the agreed skill-practice role.

Explanation: Client-directed family involvement can align with collective values and help generalize a coping routine. Consent should define what is shared and what role the aunt has rather than granting unrestricted monitoring authority.

Question 9 — Correct: C

Correct Answer: Create a specific, observable agreement linking the defined targets to the agreed privilege and review the data together.

Explanation: A clear contingency works best when target behaviors and reinforcement are observable and agreed upon. Subjective judgments about cooperation invite conflict, while indefinite removal is disproportionate and less informative.

Question 10 — Correct: C

Correct Answer: Members experience belonging and enough trust to engage honestly, including working through disagreement.

Explanation: Cohesion includes connection, commitment, and capacity for authentic group work; it does not require perpetual comfort or absence of conflict. Dependence on the leader for every rupture can limit member-to-member development.

Question 11 — Correct: B

Correct Answer: Invite each person to explain the meaning of the preferred ritual and help the family consider ways to honor more than one form of grief.

Explanation: Different mourning practices can each express respect, and exploring meanings reduces the contest over one correct ritual. Allowing one person or the majority to decide may silence other legitimate grief needs.

Question 12 — Correct: A

Correct Answer: Rehearse coping skills during gradually timed practice tests and review which responses improve performance.

Explanation: Practice under increasingly similar performance conditions allows the student to use coping and pacing skills when the relevant trigger is present. More content study does not directly address freezing and abandoned strategies under time pressure.

Question 13 — Correct: B

Correct Answer: Guide the client to observe the craving's sensations and changing intensity without acting on it, like riding a wave.

Explanation: Urge surfing teaches that cravings fluctuate and can be experienced without automatic action. Immediate distraction may sometimes help, but it would not test the client's belief that an observed craving only rises until vaping occurs.

Question 14 — Correct: C

Correct Answer: Remain with the client, activate the emergency protocol, and communicate the risk information needed for a safe evaluation.

Explanation: Current intent plus firearm access meets the stated threshold for supervised emergency intervention. Sending the client home increases access to the identified lethal means, and a promise does not adequately manage the described danger.

Question 15 — Correct: B

Correct Answer: Use the client's descriptive anchors consistently and connect each level with observable signs and coping choices.

Explanation: Reliable client-defined categories can support repeated monitoring when fine numerical distinctions are not meaningful. Preserving functional consistency is more useful than insisting on a number the client cannot apply reliably.

Question 16 — Correct: C

Correct Answer: Explore what the wished-for relationship represents while maintaining the counselor's therapeutic role.

Explanation: The statement contains important relational meaning that can be explored without enacting a parental relationship. Redirecting away loses valuable material, while claiming parent-like care blurs roles.

Question 17 — Correct: A

Correct Answer: Slow the exchange, establish a structure for factual disclosure, and help the partners examine the secrecy-criticism cycle without assigning a global label.

Explanation: A structured process addresses the financial facts and the reciprocal interaction that may maintain defensiveness and criticism. Globally labeling one partner narrows the work to blame and places the counselor in an adjudicating role.

Question 18 — Correct: B

Correct Answer: Reinforcing successive approximations that gradually move toward the target behavior.

Explanation: Shaping builds a new behavior by reinforcing achievable steps that increasingly resemble the final target. Waiting for perfect performance provides no support for acquiring the intermediate responses.

Question 19 — Correct: C

Correct Answer: Explain the concern, arrange the urgent evaluation according to policy, and remain engaged through the transfer of care.

Explanation: Severe sleep reduction, marked activation, and impaired financial judgment call for the stated urgent evaluation rather than routine outpatient pacing. A supported transfer maintains connection while moving the client to an appropriate level of assessment.

Question 20 — Correct: B

Correct Answer: Interrupt the medication advice, restate the group's scope, and redirect members toward emotional support and questions for qualified prescribers.

Explanation: Specific dose advice exceeds peer-group scope and may create medical risk, so the leader blocks it while preserving supportive discussion. Banning all hospitalization content would unnecessarily restrict clinically meaningful sharing.

Question 21 — Correct: C

Correct Answer: "I hear that I've moved to tasks before you felt understood. Let's slow down now, and I'd like to know what staying with you would look like."

Explanation: The counselor receives process feedback, acknowledges the mismatch, and asks the client to help define a more attuned response. Defending worksheets or eliminating every structured method would miss the needed balance between emotional presence and technique.

Question 22 — Correct: A

Correct Answer: Ask permission to pause, summarize the central themes heard, and collaborate on where to focus the remaining time.

Explanation: Respectful interruption and summary can organize an unproductive flow while preserving the client's role in choosing focus. Continuing unchanged ignores the client's own concern that the session lacks direction.

Question 23 — Correct: B

Correct Answer: Teach mindful walking by directing attention to steps, sensations, and gently returning focus when the mind wanders.

Explanation: Mindfulness can be practiced through movement when attention is intentionally returned to present-moment experience. Adapting the form to the client's preferences preserves the skill's function without requiring stillness.

Question 24 — Correct: B

Correct Answer: Help arrange shared supervision for tonight, support brief regulation, and identify one follow-up step for a broader safety plan.

Explanation: The caregiver needs immediate rest and practical coverage before more extensive planning. Available family support can reduce the current burden, while reassurance that wandering will not recur is unsupported.

Question 25 — Correct: A

Correct Answer: An active connection to another provider or resource, ideally with the client present and next steps confirmed.

Explanation: A warm handoff reduces the gap between referral and engagement by making an active bridge and clarifying what happens next. A passive list leaves the full navigation burden with a person already in crisis.

Question 26 — Correct: B

Correct Answer: Revisit the agreement, explore the relationship's impact on the group, and address participation options without public shaming.

Explanation: The relationship affects boundaries and group safety and therefore requires direct, respectful attention. Demanding intimate details would violate privacy, while ignoring the issue leaves the known agreement and group impact unaddressed.

Question 27 — Correct: B

Correct Answer: Redirect the conversation to the mother's goals and preferences, then facilitate family discussion about how to support them.

Explanation: Because the mother has decision-making capacity, her values should organize planning rather than be displaced by sibling conflict. Choosing for her or excluding her would undermine autonomy.

Question 28 — Correct: C

Correct Answer: "You don't need to apologize for your emotion here. What is happening for you as the tears come?"

Explanation: The response accepts the emotion and explores the client's immediate experience without comparing suffering. Premature coping would communicate that the tears need to be stopped rather than understood.

Question 29 — Correct: A

Correct Answer: Respectfully presenting a meaningful discrepancy and inviting the client to examine it.

Explanation: Therapeutic confrontation is collaborative and tentative rather than adversarial. Its purpose is to increase awareness and exploration, not to defeat resistance or warn the client forcefully.

Question 30 — Correct: C

Correct Answer: Externalize the workload problem, validate both experiences, and coach a specific request-and-response conversation about support.

Explanation: The intervention separates the problem from either partner's worth and helps them communicate needs without translating exhaustion into blame. Immediate equal task division may be medically unrealistic, while focusing on only one partner reinforces disconnection.

Question 31 — Correct: B

Correct Answer: Pause and discreetly confirm whether the client still has the level of privacy desired before proceeding.

Explanation: Telehealth privacy can change during a session, and sensitive disclosure should continue only with the client's informed awareness of the setting. An observed movement does not automatically require ending, but it warrants a respectful check.

Question 32 — Correct: A

Correct Answer: Broach how racialized and gendered expectations of strength affect her, then collaborate on forms of support that fit her values.

Explanation: The client has explicitly raised a culturally shaped expectation, making respectful broaching and client-defined support appropriate. Simply praising resilience can reproduce the pressure to endure without receiving care.

Question 33 — Correct: C

Correct Answer: Thank the client for saying so, invite requests for repetition, and adjust pace and wording while checking understanding.

Explanation: The counselor welcomes feedback and shares responsibility for accessible communication without defensiveness. Expecting passive adaptation or substituting written material does not address the live communication barrier.

Question 34 — Correct: B

Correct Answer: Being genuine and internally consistent while expressing only what serves the therapeutic relationship.

Explanation: Congruence involves authenticity rather than a false professional role, but it does not require unfiltered disclosure. Therapeutic judgment keeps the client's welfare and the relationship at the center.
